جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : نها مجدى مكرم جورجى
المحافظة : الجيزه **الحي / المدينة :** سته اكتوبر **العنوان :** ..
رقم البطاقة : 28602222500261 **رقم التليفون :** 01123083328 **تاريخ تسجيل الإستمارة :** 15-10-2026
العمر(تاريخ الميلاد) : 22-02-1986 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي العلاج المساعد لوقاية العظام للمرضي الذين يتلقوا علاج هرموني حمض الزيلودرونيك [14157] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: نها مجدى مكرم جورجى
National ID: 28602222500261

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Zoledronic acid for bone protection during hormonal therapy

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين





→ ٠١١٢٢٠٨٢٢٢٦٨

٠١٠٠٨٢٢٤٩٦٩

National ID – Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

بطريركية الاقباط الارثوذكس
كنيسة القديسة العذراء مريم بالوجوه

ST. LUKE POLYCLINIC
عيادات القديس لوقا التخصصية

ULTRASOUND REPORT

Name: *Noha Magdy* *Date : 25/08/ 2026*

Bilateral Breast Sonography revealed :

Breast composition: **Prominent fibroglandular tissue element.**

Findings:

- **Breast parenchyma:**

Left breast shows large ill defined hyoechoic mass lesion occupying all breast quadrants with multiple areas of internal vasculature and no definite internal fluid collection

No definite clusters of micro calcification.

No dilated retro-areolar lactiferous ducts.

Right breast shows no definite solid or cystic lesions

- **Axilla:**

Bilateral multiple enlarged axillary lymph nodes with preserved hilum and hypertrophied cortex measuring about 12 x 11 mm

Opinion:

- ***Large left breast mass lesion (BIRADS IVa)***

Best Regards,
Dr.Manal Maher

٢٨ شارع قبة الهـــواء - شيكولانـــي - شبرا مصـــر
ت : ٢٢٣٩٢٦٢١ - ٢٢٣١٩٥٢٢ - ٢٢٣١٩٥٣٣ ٠١٢٧٧٣٢٦٤٠٦
St.Lukepolyclinic

Sonography
[[DOCTYPE:SONOGRAPHY]]

Noha Magdy Makram George
2026-09-10
Prof.Dr. **no doctor**
Goustave Roussey International Egypt

nt, for biopsy. 50 %

ion.

psy. 98-100%

ven malignant



Noha Magdy Makram George
2026-09-10
Prof.Dr. **no doctor**
Goustave Roussey International Egypt

or biopsy. 50 %

98-100%

nalignant



[[DOCTYPE:SONOGRAPHY]]
Sonography

أ.د. أحمد كمال الدرى
أ.د. محمد أبو الهدى درويش
أ.د. عمر حسين عمر
أ.د. وحيد حسين طنطاوى
أساتذه الأشعة التشخيصية
كلية الطب – جامعة عين شمس

مركز مصر للأشعة
MISR RADIOLOGY CENTER MRC

MRS / NOHA MAGDY MAKRAM	**PATIENT ID: 1555006**
DEAR SIR	**STUDY DATE: 29/08/2026**

BILATERAL 3D DIGITAL BREAST TOMOSYTHESIS WITH HIGH RESOLUTION ULTRASOUND, REVEALED:-

- Right breast shows normal skin, nipple, areola & subcutaneous tissue.
- Left breast shows skin thickening and subcutaneous oedema.
- Both breasts show predominant fibroglandular tissue with nodularity impressive of fibroadenosis (ACR-C).
- Left breast ill defined retro-areolar irregular mass with irregular spiculated border.
- Right axillary lymphadenopathy with preserved fatty hilum likely reactionary.

High resolution ultrasonography:

- Sonographic examination confirmed the mammographic findings and revealed left breast ill defined retro-areolar solid irregular hypoechoic mass with spiculated border measuring 5.6 X 4 cm, it shows surrounding satellites, largest at 10 O'clock 5 cm from the nipple 10 X 9 mm, with internal vascularity, impressive of highly suspicious lesion with left axillary left nodes rounded in shape with lost fatty hilum, the largest measuring about 10 X 7 mm, another left axillary ill defined hypoechoic solid irregular lesion with spiculated border measuring about 11 X 8 mm is noted.
- Bilateral scattered simple cysts ranging 2 – 5 mm.

IMPRESSION:-

- **Left breast and left axillary highly suspicious lesions, for biopsy and histopathological correlation (BIRADS 4c).**
- **Bilateral scattered simple cysts.**

MUCH OBLIGED
A. PROF. DR. MAI MOKHTAR
PROF. DR. MOHAMED ABOU EL HODA
R.A

[[DOCTYPE:SONOGRAPHY]]
Sonography

مركز مصر للأشعة
MISR RADIOLOGY CENTER
MRC

ا.د. أحمد كمال الدرى
ا.د. محمد أبو الهدى درويش
ا.د. عمر حسين عمر
ا.د. وحيد حسين طنطاوى
أساتذه الأشعة التشخيصية
كلية الطب – جامعة عين شمس

BI-RADS 0	Need Additional Imaging Evaluation and/or Prior Mammograms For Comparison:
BI-RADS 1	Negative:
BI-RADS 2	Benign Finding:
BI-RADS 3	Probably Benign Finding – Initial Short-Interval Follow-Up Suggested: less than a 2% risk of malignancy.
BI-RADS 4	Suspicious Abnormality – Biopsy Should Be Considered: wide range of probability of malignancy (2 – 95%).
BI-RADS 4 Category 4a	low suspicion for malignancy
BI-RADS 4 Category 4b	moderate suspicion for malignancy
BI-RADS 4 Category 4c	high suspicion for malignancy
BI-RADS 5	Highly Suggestive of Malignancy. Appropriate Action Should Be Taken: with a >95% likelihood of malignancy.
BI-RADS 6	Known Biopsy Proven Malignancy. Appropriate Action Should Be Taken

ACR Classification of breast density

- a = Almost entirely fatty
- b = Scattered areas of fibro-glandular density
- c = Heterogeneously dense, which might obscure small masses.
- d = Extremely dense, decreasing the sensitivity of the mammogram.

[[DOCTYPE:PATHOLOGY_REPORT]]
Pathology Report

GUSTAVE ROUSSY INTERNATIONAL EGYPT
Gustave Roussy International-Egypt- Pathology Lab
جوستاف روسي انترناشونال – مصر معمل الباثولوجي
جوستاف روسي

Patient Name	نها مجدى مكرم	**Case No.**	**361861**
Sex/Age	**Female 40 Y**	**Patient No.**	**26571**
Registered Date	**01/9/2026 10:05:47 AM**	**Referred By Dr.**	**Interventional Radiology**
Validation Date	**21/9/2026 11:22:15 AM**	**Branch**	**Gustave Roussy**
Reporting Date	**21/9/2026 11:24:47 AM**	**Pathology No.**	**2814/26**

Pathology Report

Clinical Data: Left breast mass (BIRADS 4a).

Nature of Specimen: Tru-cut biopsy, collected on 01/09/2026. Cold ischemia time is 20 seconds.

Gross Examination:
- Three grey, white tissue cores, ranging in size between 0.8 cm & 1.2 cm long, totally submitted.

Technique:
- Sections prepared from paraffin block for immune-staining using Benchmark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination:
- **Histologic type:** Invasive duct carcinoma with focal lobular component.
- **Histologic grade:** 2 (Differentiation 3, pleomorphism 2, mitosis 1).
- **Ductal carcinoma in situ:** Absent.
- **Microcalcification:** Absent.
- **Angioinvasion:** Absent.
- **Perineural invasion:** Absent.
- **Tumor infiltrating lymphocytes (TILs):** 1%.

Results:
*** IPT (All controls showed appropriate reactivity):**
- ER: Strong nuclear reaction in 80% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- Her2: Faint incomplete membrane reaction in more than 10% of tumor cells (score 1+).

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

GUSTAVE ROUSSY INTERNATIONAL
Gustave Roussy International-Egypt- Pathology Lab
جوستاف روسى انترناشونال – مصر معمل الباثولوجي
جوستاف روسي

Patient Name	نها مجدى مكرم	**Case No.**	**361861**
Sex/Age	**Female 40 Y**	**Patient No.**	**26571**
Registered Date	**01/9/2026 10:05:47 AM**	**Referred By Dr.**	**Interventional Radiology**
Validation Date	**21/9/2026 11:22:15 AM**	**Branch**	**Gustave Roussy**
Reporting Date	**21/9/2026 11:24:47 AM**	**Pathology No.**	**2814/26**

- Ki67%: 60%.

Conclusion:

Left breast mass, Tru-cut biopsy:

- Invasive duct carcinoma with focal lobular component, Grade 2.

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 8/8).
- Her2: Low (score 1+).
- Ki67%: 60%.

***Scoring system: "ER, PR"*

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.

** ***Scoring system: "Her2"***

Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of

the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature:
Prof. Dr. Maha Guimei, MD, PhD.

[[DOCTYPE:MAMMOGRAM]]
Mammogram

جوستاف روسي انترناشونال

Patient's Name: Noha Magdy Makram George
Date: 2026-09-10
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: left breast mass with nipple retraction

Right breast.

- Heterogenosly dense breast . (ACR c)
- No speculations, masses, asymmetry, distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Solitary Indeterminate right axillary LN 1.3 cm x 0.9cm with eccentric hilum and cortical thickness =6 mm .

Left breast

- Heterogenously dense breast (ACR)
- non circumscribed irregular speculated dense mass occupying most of the central retroareolar region measuring about 8m x 4.3 cm with nipple retraction .
- **Ultrasound**
- the left breast show diffuse infiltrative process in the form of large irregular heterogenous hypoechoic non mass area involving the UO breast measuring about 6cm x 3.5 cm another 12 o'
  clock para areolar mass is seen measuring 2x1.4 cm
- the UIQ of the left breast show heterogenous hypoechoic non mas area extending in the para areolar region between 10 o'clock and 12 o'clock .
- Thickened skin reaching 10 mm with interstitial and subdermal edema and retracted nipple .
- Multiple (more than three) pathological axillary LNs are noted at levels 1, 2 & 3, one showing irregular outline ?? extra-capsular extension, largest measured14 x9 mm with lost hilum.
- pathological supra-clavicular lymph node area seen .
- Pathological internal mammary lymph node is seen measring 1.2 cm x1.2 cm .

Conclusion: -

- ***Right breast (BIRADS 1): normal with indeterminate axillary LN .***
- ***Left breast (BIRADS 5): Locally advanced malignant process with ipsilateral multilevel pathological axillary lymph nodes level 1,2 and 3 pathological supraclavicular and internal mammary LNs .***

Best Regards,
DR. Dina Sayed , M.Sc,CUDBIT

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with recommendations:

Category & recommendations	Suspected Cancer Risk
BIRADS 0: Non-informative study.	
BIRADS 1: Normal study, routing screening	0 %
BIRADS 2: Benign findings, routine screening	0 %
BIRADS 3: Probably benign, for close follow up.	2 %

[[DOCTYPE:MAMMOGRAM]]
Mammogram

Patient's Name:	**Noha Magdy Makram George**
Date:	**2026-09-10**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

BIRADS 4: Probably malignant, for biopsy.	50 %
A: mild suspicion.	
B: moderate suspicion.	
C: high suspicion.	
BIRADS 5: Malignant, for biopsy.	98-100%

BIRADS 6: Pathologically proven malignant

Noha Magdy Makram George
2026-09-10
Prof.Dr. **no doctor**
Goustave Roussey International Egypt

nt, for biopsy. 50 %

ion.

psy. 98-100%

ven malignant



Noha Magdy Makram George
2026-09-10
Prof.Dr. **no doctor**
Goustave Roussey International Egypt

or biopsy. 50 %

98-100%

nalignant



ا.د. أحمد كمال الدرى
ا.د. محمد أبو الهدى درويش
ا.د. عمر حسين عمر
ا.د. وحيد حسين طنطاوى
أساتذه الأشعة التشخيصية
كلية الطب – جامعة عين شمس

MRS / NOHA MAGDY MAKRAM	**PATIENT ID: 1555006**
DEAR SIR	**STUDY DATE: 29/08/2026**

BILATERAL 3D DIGITAL BREAST TOMOSYTHESIS WITH HIGH RESOLUTION ULTRASOUND, REVEALED:-

- Right breast shows normal skin, nipple, areola & subcutaneous tissue.
- Left breast shows skin thickening and subcutaneous oedema.
- Both breasts show predominant fibroglandular tissue with nodularity impressive of fibroadenosis (ACR-C).
- Left breast ill defined retro-areolar irregular mass with irregular spiculated border.
- Right axillary lymphadenopathy with preserved fatty hilum likely reactionary.

High resolution ultrasonography:

- Sonographic examination confirmed the mammographic findings and revealed left breast ill defined retro-areolar solid irregular hypoechoic mass with spiculated border measuring 5.6 X 4 cm, it shows surrounding satellites, largest at 10 O'clock 5 cm from the nipple 10 X 9 mm, with internal vascularity, impressive of highly suspicious lesion with left axillary left nodes rounded in shape with lost fatty hilum, the largest measuring about 10 X 7 mm, another left axillary ill defined hypoechoic solid irregular lesion with spiculated border measuring about 11 X 8 mm is noted.
- Bilateral scattered simple cysts ranging 2 – 5 mm.

IMPRESSION:-

- **Left breast and left axillary highly suspicious lesions, for biopsy and histopathological correlation (BIRADS 4c).**
- **Bilateral scattered simple cysts.**

MUCH OBLIGED
A. PROF. DR. MAI MOKHTAR
PROF. DR. MOHAMED ABOU EL HODA
R.A

أ.د. أحمد كمال الدرى
أ.د. محمد أبو الهدى درويش
أ.د. عمر حسين عمر
أ.د. وحيد حسين طنطاوى
أساتذه الأشعة التشخيصية
كلية الطب – جامعة عين شمس

BI-RADS 0	Need Additional Imaging Evaluation and/or Prior Mammograms For Comparison:
BI-RADS 1	Negative:
BI-RADS 2	Benign Finding:
BI-RADS 3	Probably Benign Finding – Initial Short-Interval Follow-Up Suggested: less than a 2% risk of malignancy.
BI-RADS 4	Suspicious Abnormality – Biopsy Should Be Considered: wide range of probability of malignancy (2 – 95%).
BI-RADS 4 Category 4a	low suspicion for malignancy
BI-RADS 4 Category 4b	moderate suspicion for malignancy
BI-RADS 4 Category 4c	high suspicion for malignancy
BI-RADS 5	Highly Suggestive of Malignancy. Appropriate Action Should Be Taken: with a >95% likelihood of malignancy.
BI-RADS 6	Known Biopsy Proven Malignancy. Appropriate Action Should Be Taken

ACR Classification of breast density

- a = Almost entirely fatty
- b = Scattered areas of fibro-glandular density
- c = Heterogeneously dense, which might obscure small masses.
- d = Extremely dense, decreasing the sensitivity of the mammogram.

GUSTAVE ROUSSY INTERNATIONAL EGYPT
Gustave Roussy International-Egypt- Pathology Lab
جوستاف روسى انترناشونال – مصر معمل الباثولوجى
جوستاف روسي

Patient Name	نها مجدى مكرم	**Case No.**	**361861**
Sex/Age	**Female 40 Y**	**Patient No.**	**26571**
Registered Date	**01/9/2026 10:05:47 AM**	**Referred By Dr.**	**Interventional Radiology**
Validation Date	**21/9/2026 11:22:15 AM**	**Branch**	**Gustave Roussy**
Reporting Date	**21/9/2026 11:24:47 AM**	**Pathology No.**	**2814/26**

Pathology Report

Clinical Data: Left breast mass (BIRADS 4a).

Nature of Specimen: Tru-cut biopsy, collected on 01/09/2026. Cold ischemia time is 20 seconds.

Gross Examination:
- Three grey, white tissue cores, ranging in size between 0.8 cm & 1.2 cm long, totally submitted.

Technique:
- Sections prepared from paraffin block for immune-staining using Benchmark ULTRA system for;
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination:
- **Histologic type:** Invasive duct carcinoma with focal lobular component.
- **Histologic grade:** 2 (Differentiation 3, pleomorphism 2, mitosis 1).
- **Ductal carcinoma in situ:** Absent.
- **Microcalcification:** Absent.
- **Angioinvasion:** Absent.
- **Perineural invasion:** Absent.
- **Tumor infiltrating lymphocytes (TILs):** 1%.

Results:
*** IPT (All controls showed appropriate reactivity):**
- ER: Strong nuclear reaction in 80% of tumor cells (score 8/8).
- PR: Strong nuclear reaction in 90% of tumor cells (score 8/8).
- Her2: Faint incomplete membrane reaction in more than 10% of tumor cells (score 1+).

GUSTAVE ROUSSY INTERNATIONAL HOSPITAL
Gustave Roussy International-Egypt- Pathology Lab
جوستاف روسى انترناشونال – مصر معمل الباثولوجى
جوستاف روسي

Patient Name	نها مجدى مكرم	**Case No.**	**361861**
Sex/Age	**Female 40 Y**	**Patient No.**	**26571**
Registered Date	**01/9/2026 10:05:47 AM**	**Referred By Dr.**	**Interventional Radiology**
Validation Date	**21/9/2026 11:22:15 AM**	**Branch**	**Gustave Roussy**
Reporting Date	**21/9/2026 11:24:47 AM**	**Pathology No.**	**2814/26**

- Ki67%: 60%.

Conclusion:

Left breast mass, Tru-cut biopsy:

- Invasive duct carcinoma with focal lobular component, Grade 2.

IPT:

- ER: Positive (score 8/8).
- PR: Positive (score 8/8).
- Her2: Low (score 1+).
- Ki67%: 60%.

***Scoring system: "ER, PR"*

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.

** *Scoring system: "Her2"*

Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.

IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of

the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature:
Prof. Dr. Maha Guimei, MD, PhD.

Patient's Name:	**Noha Magdy Makram George**
Date:	**2026-09-10**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: left breast mass with nipple retraction

Right breast.

- Heterogenosly dense breast . (ACR c)
- No speculations, masses, asymmetry, distortion or suspicious microcalcifications.
- **Ultrasound** demonstrated no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Solitary Indeterminate right axillary LN 1.3 cm x 0.9cm with eccentric hilum and cortical thickness =6 mm .

Left breast

- Heterogenously dense breast (ACR)
- non circumscribed irregular speculated dense mass occupying most of the central retroareolar region measuring about 8m x 4.3 cm with nipple retraction .
- **Ultrasound**
- the left breast show diffuse infiltrative process in the form of large irregular heterogenous hypoechoic non mass area involving the UO breast measuring about 6cm x 3.5 cm another 12 o'
  clock para areolar mass is seen measuring 2x1.4 cm
- the UIQ of the left breast show heterogenous hypoechoic non mas area extending in the para areolar region between 10 o'clock and 12 o'clock .
- Thickened skin reaching 10 mm with interstitial and subdermal edema and retracted nipple .
- Multiple (more than three) pathological axillary LNs are noted at levels 1, 2 & 3, one showing irregular outline ?? extra-capsular extension, largest measured14 x9 mm with lost hilum.
- pathological supra-clavicular lymph node area seen .
- Pathological internal mammary lymph node is seen measring 1.2 cm x1.2 cm .

Conclusion: -

- ***Right breast (BIRADS 1): normal with indeterminate axillary LN .***
- ***Left breast (BIRADS 5): Locally advanced malignant process with ipsilateral multilevel pathological axillary lymph nodes level 1,2 and 3 pathological supraclavicular and internal mammary LNs .***

Best Regards,

DR. Dina Sayed , M.Sc,CUDBIT

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with recommendations:

Category & recommendations	Suspected Cancer Risk
BIRADS 0: Non-informative study.	
BIRADS 1: Normal study, routing screening	0 %
BIRADS 2: Benign findings, routine screening	0 %
BIRADS 3: Probably benign, for close follow up.	2 %

Patient's Name:	**Noha Magdy Makram George**
Date:	**2026-09-10**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

BIRADS 4: Probably malignant, for biopsy.	50 %
A: mild suspicion.	
B: moderate suspicion.	
C: high suspicion.	
BIRADS 5: Malignant, for biopsy.	98-100%

BIRADS 6: Pathologically proven malignant

[[DOCTYPE:MDT_FORM]]
MDT Form

جوستاف روسي

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

البنك المصرية من أجل صحة مصر

10/11/2026

MDT FORM

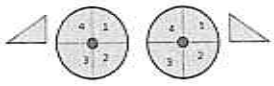
Name:	نها مجدى مكرم جورجى	**MRN:**	14105
Age:	41 Y	**ID:**	28602222500261
Menopausal Sta	pre	**Phone:***	01123083328
NRS SCORE:		**Marital Status:**	
BMI:		**Family History:**	
Gender			
CO Morbidity:		**Reason Of MDT:**	new case
Surgical History:*	NAD	**ECOG Performance Status**	○ 0 ◉ 1 ○ 2 ○ 3 ○ 4

Clinical Data:

BX: IDC GII

ER: 8/8 **PR:** 8/8 **HER2:** +1 **KI67:** 60 %

R L



T N M

History:

Sonomammography:

BSM > Left breast locally advanced malignant process with ipsilateral multilevel path ax LN level 1,2 and 3 path supraclav + IMLNs
ct4n N2 M1

Biomarker/Molecular

Mets Workup:

CT CAP :
-left breast parenchymal distortion with soft tissue density lesions and edema + skin thickening Left axillary globular shape soft tissue lesion 2,5 cm -Bilateral lungs multiple metastatic nodules Right middle lung lobe consolidative masslike lesion 5 cm
- Right mild pleural effusion
-Sternal, vertebrae and pelvis bones multiple sclerotic metastatic bony lesion

Recommendations:

for AI+ zoladex + CDK4/6 I , zometa q3 months

Right Side				Left Side			
Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

Clinical Staging:

ct4n N2 M1

Pathological Staging:

MDT Outcome & Plan

for AI+ zoladex , zometa q3 months
TRIAL OF CDK4/6 FROM ngo

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د حنان عزت	د احمد نجيب		